ENT Examination — OSCE Checklist

Deakin MD2 · BB4 · Time: 8 min (+2 min reading)

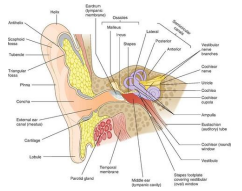
Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Ears (inspect → palpate → otoscopy → hearing) → Nose (inspect → airflow → rhinoscopy → sinuses) → Throat (mouth → tonsils → neck)**. *Deakin quick reference order. Examine the* **good ear first**.

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain the exam and gain consent; ask about pain
☐	Ask about ear, nose and throat symptoms; position seated, at your eye level
☐	Equipment: otoscope with range of speculae, 512 Hz tuning fork, pen torch, tongue depressor, nasal speculum (or otoscope with wide speculum), gloves

2. Ears

	Step	What to do / say	Abnormal
Ear — inspection (external ear)			
☐	Pinna	Size, shape, deformity; trauma (haematoma)	Cauliflower ear
☐	Pre-auricular region	Cartilage tags, sinuses or pits	Syndromes with hearing loss
☐	Skin	Suspicious lesions (SCC, BCC), skin conditions of pinna and canal	Neoplasia, eczema
☐	Scars	Pre- or post-auricular	Previous surgery
☐	Hearing aids	Note and remove before otoscopy	
☐	Discharge	Mucopurulent, serous, blood-stained	Otitis externa / media, perforation



Anatomy of the ear



Otitis externa



Squamous cell carcinoma of the pinna



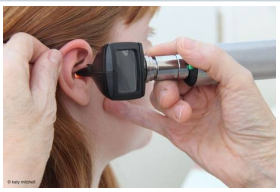
Preauricular sinus

Ear — palpation			
☐	Tragus and pinna	Gently move — tenderness?	Tender = otitis externa
☐	Mastoid	Palpate behind the ear	Tender = mastoiditis (with otitis media)

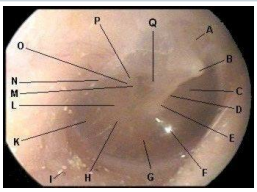


Acute mastoiditis

Ear — otoscopy			
☐	Speculum	Largest that fits comfortably	
☐	Grip	Hold otoscope like a pen; hand braced against patient's head	Prevents injury if head moves
☐	Straighten canal	Adult: pull pinna up and back; child: pull back only	
☐	Canal	Inflamed? Wax? Discharge? Foreign body?	Otitis externa
☐	Tympanic membrane	Colour, light reflex, short process and handle of malleus, umbo, pars flaccida; move to see whole TM	Bulging, retraction, perforation, grommet



Holding the otoscope — hand braced on head



Right tympanic membrane — landmarks



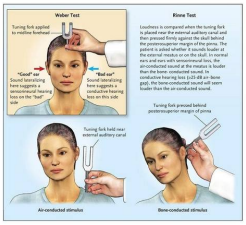
Grommet (ventilation tube)



Common ear drum findings

Ear — hearing			
☐	Whisper test	Mask other ear (rub tragus / fingers); whisper numbers; increase volume if needed; each side	Formal audiology if abnormal
☐	Weber (512 Hz)	Strike fork; base on vertex or midline forehead: "Louder on one side or the middle?"	Lateralises (see table)

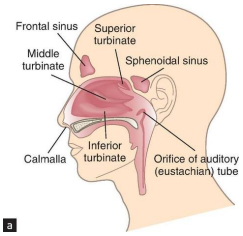
☐	Rinne (512 Hz)	Base on mastoid until no longer heard, then prongs beside the ear: "Can you hear it now?"	Positive (AC > BC) = normal



Weber and Rinne tests

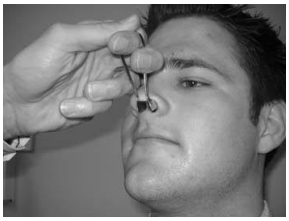
3. Nose and sinuses

	Step	What to do / say	Abnormal
Nose — external inspection			
☐	Front and side	Swelling, deformity, scars or creases, redness / skin disease	Trauma, rhinophyma, BCC
☐	From above	Patient tilts head back; look down the bridge	Deviated nose
☐	Discharge	Rhinorrhoea, crusting; unilateral?	Unilateral offensive in a child = foreign body
☐	Nasal bones	Gently palpate bridge	Tenderness, crepitus, step = fracture



Nasal and sinus anatomy

Nose — airflow			
☐	Misting test	Cold metal tongue depressor under nose as patient breathes out	Condensation under both nostrils = airflow
☐	Sniff test	Occlude one nostril, ask patient to sniff; repeat	Unilateral obstruction
Nose — anterior rhinoscopy (torch ± speculum)			
☐	Without speculum	Tip nose up with thumb; look into the anterior nares with torch	
☐	With speculum / otoscope	Gently open; look straight back, not up	
☐	Septum	Straight or deviated? Bleeding points, clots, crusting, perforation; septal haematoma	Little's area = common epistaxis site †; haematoma = urgent drainage
☐	Turbinates	Inflammation, swelling, colour	Rhinitis (pale, boggy = allergic †)
☐	Polyps*	Grey, glistening, insensate †	Chronic rhinosinusitis



Nasal speculum examination

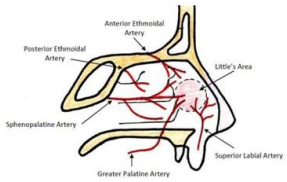


Figure 1. Blood supply to the nasal septum

Blood supply of the septum — Little's area

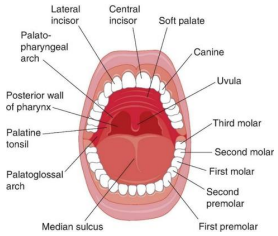


Nasal polyp

Sinuses			
☐	Palpate	Frontal (above brow) and maxillary (cheek) sinuses	Tenderness = sinusitis
☐	Transilluminate*	Pen torch in a dark room (demonstrated in session)	Reduced = fluid / mass
☐	Note	Ethmoid and sphenoid sinuses cannot be examined physically	

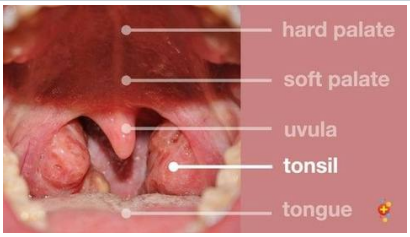
4. Throat and neck

	Step	What to do / say	Abnormal
Mouth — oral cavity (bright pen torch, mouth wide open)			
☐	Lips and mucosa	Ulcers, lesions, colour	Aphthous ulcers, candida, malignancy †
☐	Dentition	Comment on teeth and gums	Dental caries, abscess
☐	Tongue	Appearance; ask to raise tongue to check underneath if indicated	Lesions, floor-of-mouth swelling



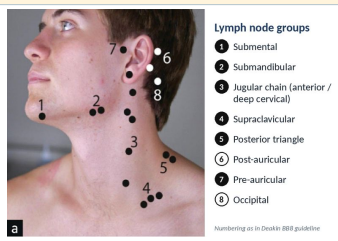
Anatomy of the mouth

Throat — oropharynx			
☐	"Say ahh"	Mouth wide, tongue protruded	Palate rises symmetrically; uvula central †
☐	Tongue depressor	If tonsils not seen: press middle third of tongue, not the back †	
☐	Tonsils	Size, symmetry, inflammation, exudate	Tonsillitis; asymmetry = quinsy or malignancy †
☐	Pharynx	Swelling, inflammation, exudate, post-nasal drip	



Palate, uvula and tonsils

Neck (if clinically indicated)			
☐	Lymph nodes	From behind: submental, submandibular, tonsillar, cervical chain, supraclavicular, pre/post-auricular, occipital	Infection, malignancy
☐	Parotids*	Palpate in front of ear and over angle of jaw	Parotitis, tumour



Cervical lymph node groups

5. Closing and presenting

☐	Thank patient; clean otoscope speculum; dispose of tongue depressor; wash hands
☐	Present: "Pinnae normal and non-tender; canals clear; both TMs intact and pearly grey; whisper test normal, Weber central, Rinne positive bilaterally. No nasal deformity, airways patent, septum midline, no sinus tenderness. Good dentition; tonsils symmetrical without exudate; uvula central; no cervical lymphadenopathy."
☐	Completion: "I would arrange formal audiometry and consider nasendoscopy or a throat swab if indicated."

6. Quick reference

Weber / Rinne (Deakin table)	Weber central	Weber lateralises LEFT	Weber lateralises RIGHT
Rinne AC > BC both ears	Normal	Sensorineural loss RIGHT	Sensorineural loss LEFT
Rinne BC > AC left	—	Conductive loss LEFT	Combined loss
Rinne BC > AC right	—	Combined loss	Conductive loss RIGHT

TM appearance †	Think of
Pearly grey, light reflex	Normal
Red, bulging, lost landmarks	Acute otitis media
Dull, retracted ± fluid level / bubbles	Otitis media with effusion
Hole in TM	Perforation
Small tube in TM	Grommet (not a foreign body)
Swollen canal, debris, tender tragus	Otitis externa

- **Conductive** loss: Weber goes TO the bad ear, Rinne negative (BC > AC).
- **Sensorineural** loss: Weber goes AWAY from the bad ear, Rinne positive (both reduced).
- **False-negative Rinne**: a dead ear hears BC louder via the other cochlea — Weber clarifies.
- **Quinsy †**: unilateral tonsil swelling, uvula deviated away, trismus, "hot potato" voice — urgent ENT.
- **Airway red flags †**: stridor, drooling, can't swallow saliva — don't use a tongue depressor; call for help.